

NanoGT Match Report: Fabry disease

Disease: Fabry disease (ORPHA:324)

Primary gene: GLA

Gene CDS: 1290 bp

Inheritance: X-linked dominant

Target tissues: liver, kidney, heart, CNS

Top 5 GT Precedent Matches

Rank	Program	Vector	Score	Confidence	Approval
1	Libmeldy	LV	8.7/10	[GREEN] High	approved
2	ST-920	AAV2/6	8.6/10	[GREEN] High	phase1/2
3	RGX-121	AAV9	8.4/10	[GREEN] High	phase3
4	Hemgenix	AAV5	8.3/10	[GREEN] High	approved
5	Roctavian	AAV5	8.3/10	[GREEN] High	approved

Match #1: Libmeldy

Precedent disease: Metachromatic leukodystrophy

Vector: LV

Tissue target: hematopoietic/CNS

Composite score: 8.7 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	1.50	2.0	Vector naturally reaches disease target tissue
Protein class	2.00	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	2.00	2.0	Same or related biological pathway
Inheritance compatibility	0.70	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?

Dimension	Score	Max	What it measures
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.90	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
TOTAL (normalised)	8.67	10.0	Raw sum / 18 × 10

Rationale

- Gene CDS 1290bp / cargo 8000bp (16% utilized)
- Precedent target match: cns
- Both lysosomal proteins — cross-correction likely
- LOF inheritance — compatible for gene replacement
- Pathway match: leukodystrophy
- Approval status: approved
- Vector immunogenicity (LV): low (~2%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted lysosomal enzyme — maximum cross-correction via M6P receptor pathway; high therapeutic efficiency; substantially lower required transduction rate
- Immune privilege: high privilege — blood-brain barrier severely limits T-cell access; durable expression expected
- Promoter availability: ApoE/haAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs

Match #2: ST-920

Precedent disease: Fabry disease

Vector: AAV2/6

Tissue target: liver

Composite score: 8.6 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	2.00	2.0	Vector naturally reaches disease target tissue

Dimension	Score	Max	What it measures
Protein class	2.00	2.0	Same se-creted/lysosomal/membrane/intracellular class
Pathway similarity	2.00	2.0	Same or related biological pathway
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	0.50	1.0	Regulatory approval / trial stage
Immunogenicity	1.50	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.90	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
TOTAL (normalised)	8.56	10.0	Raw sum / 18 × 10

Rationale

- Gene CDS 1290bp / cargo 4700bp (27% utilized)
- Vector tropism plus precedent target match: liver
- Both lysosomal proteins — cross-correction likely
- Inheritance match (X-linked dominant <-> XL)
- Pathway match: lysosomal_storage
- Approval status: phase1/2
- Vector immunogenicity (AAV2/6): moderate (~17%) — significant proportion may require pre-screening or exclusion
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted lysosomal enzyme — maximum cross-correction via M6P receptor pathway; high therapeutic efficiency; substantially lower required transduction rate
- Immune privilege: high privilege — blood-brain barrier severely limits T-cell access; durable expression expected
- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs

Match #3: RGX-121

Precedent disease: Mucopolysaccharidosis type II

Vector: AAV9

Tissue target: CNS/liver
Composite score: 8.4 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	2.00	2.0	Vector naturally reaches disease target tissue
Protein class	2.00	2.0	Same se-creted/lysosomal/membrane/intracellular class
Pathway similarity	2.00	2.0	Same or related biological pathway
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	0.80	1.0	Regulatory approval / trial stage
Immunogenicity	1.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.90	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
TOTAL (normalised)	8.44	10.0	Raw sum / 18 × 10

Rationale

- Gene CDS 1290bp / cargo 4700bp (27% utilized)
- Vector tropism plus precedent target match: cns, liver
- Both lysosomal proteins — cross-correction likely
- Inheritance match (X-linked dominant <-> XL)
- Pathway match: lysosomal_storage
- Approval status: phase3
- Vector immunogenicity (AAV9): high (~22%) — substantial patient exclusion expected; immunodepletion protocols may be needed
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted lysosomal enzyme — maximum cross-correction via M6P receptor pathway; high therapeutic efficiency; substantially lower required transduction rate
- Immune privilege: high privilege — blood-brain barrier severely limits T-cell access; durable expression expected

- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs

Match #4: Hemgenix

Precedent disease: Hemophilia B

Vector: AAV5

Tissue target: liver

Composite score: 8.3 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	2.00	2.0	Vector naturally reaches disease target tissue
Protein class	2.00	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	0.50	2.0	Same or related biological pathway
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.90	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue
TOTAL (normalised)	8.28	10.0	Raw sum / 18 × 10

Rationale

- Gene CDS 1290bp / cargo 4700bp (27% utilized)
- Vector tropism plus precedent target match: liver
- Both secreted proteins — systemic delivery viable
- Inheritance match (X-linked dominant <-> XL)
- Different pathway (lysosomal_storage vs coagulation)

- Approval status: approved
- Vector immunogenicity (AAV5): low (~9%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted lysosomal enzyme — maximum cross-correction via M6P receptor pathway; high therapeutic efficiency; substantially lower required transduction rate
- Immune privilege: high privilege — blood-brain barrier severely limits T-cell access; durable expression expected
- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
- Route of administration: IV systemic — established, minimally invasive; used in all hepatic GT programs

Match #5: Roctavian

Precedent disease: Hemophilia A

Vector: AAV5

Tissue target: liver

Composite score: 8.3 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	2.00	2.0	Vector naturally reaches disease target tissue
Protein class	2.00	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	0.50	2.0	Same or related biological pathway
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	0.90	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	1.00	1.0	Established delivery route to target tissue

Dimension	Score	Max	What it measures
TOTAL (normalised)	8.28	10.0	Raw sum / 18 × 10

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- Gene CDS 1290bp / cargo 4700bp (27% utilized)
- Vector tropism plus precedent target match: liver
- Both secreted proteins — systemic delivery viable
- Inheritance match (X-linked dominant <-> XL)
- Different pathway (lysosomal_storage vs coagulation)
- Approval status: approved
- Vector immunogenicity (AAV5): low (~9%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted lysosomal enzyme — maximum cross-correction via M6P receptor pathway; high therapeutic efficiency; substantially lower required transduction rate
- Immune privilege: high privilege — blood-brain barrier severely limits T-cell access; durable expression expected
- Promoter availability: ApoE/hAAT, TBG, transthyretin, albumin — extensively validated; used in Hemgenix, Roctavian, DTX301
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